

## Comment on HHS Health Sector AI RFI

### Submitted by: Basys.ai (Company)

Basys.ai appreciates the opportunity to respond to the Department of Health and Human Services' Request for Information regarding accelerating the adoption and use of artificial intelligence in clinical care.

Basys.ai is an AI-powered healthcare technology company focused on automating prior authorization, utilization management, and medical review workflows for health plans and government contractors. Through our work with health plans, CMS-aligned programs, and healthcare administrators, we have observed firsthand the operational barriers, regulatory friction, and workflow inefficiencies that slow responsible AI adoption in clinical environments.

Our responses below are grounded in real-world implementation experience, including reducing administrative burden in CMS-related review programs and participation in federal innovation initiatives. We have also included some of our publications and announcements in the references at the end of this document, including a reference for the [Healthcare Moonshot publication](#) that principally guides our response to the RFI.

## 1. Biggest Barriers to AI Innovation and Adoption in Clinical Care

### a. Workflow Fragmentation and Legacy Infrastructure

Clinical decision-making remains embedded in siloed EHR systems, PDF documents, manual review queues, and disconnected policy repositories. AI cannot scale without structured, interoperable inputs.

### b. Regulatory Ambiguity for Non-Device AI

For administrative-clinical AI (e.g., medical necessity review, coverage validation), governance expectations are unclear. Organizations are uncertain about liability, documentation, and audit standards.

### c. Misaligned Incentives in Fee-for-Service Models

AI that reduces unnecessary utilization or administrative waste may not align with legacy reimbursement incentives.

### d. Trust and Transparency Gaps

Health plans and providers require explainability, audit trails, and clinician override pathways. AI tools lacking structured transparency slow adoption.

**Upshot:** In our experience, we have seen the biggest barriers to the adoption of AI are overconfidence and a lack of transparency in AI implementation. We have addressed this challenge [here](#) and also discussed a resolution framework.

## 2. Regulatory and Payment Policy Changes HHS Should Prioritize From Our Wider Payer Learning/Perspective

### Regulatory

#### a. Establish Governance Framework for Non-Device Clinical AI

Clarify expectations for AI used in clinical-adjacent decision support (e.g., prior authorization, utilization review) distinct from FDA-regulated devices.

#### b. Standardized Documentation and Audit Requirements

Provide guidance under HIPAA (45 CFR Parts 160 & 164) and related CMS program integrity frameworks to define acceptable AI audit logs, traceability standards, and human-in-the-loop requirements.

#### c. AI Transparency Safe Harbor

Create regulatory safe harbors for AI systems that meet defined transparency, documentation, and clinician oversight thresholds.

### Payment Policy

#### a. Incentivize Administrative Efficiency in Value-Based Models

Align CMS payment programs so that AI-driven reductions in administrative burden and inappropriate utilization are financially rewarded.

#### **b. Recognize AI-Enabled Care Pathway Optimization**

Allow AI-supported clinical triage and alternative care pathway recommendations to be integrated into reimbursement modernization efforts.

### **3. Novel Legal and Implementation Issues (Non-Medical Devices)**

#### **a. Liability Allocation**

Clear delineation is needed between AI tool vendors, health plans, and clinical users when AI informs but does not autonomously decide.

#### **b. Auditability and Appeals Governance**

Administrative AI must produce defensible rationales for coverage decisions, especially in Medicare and Medicaid programs.

#### **c. Data Provenance and Model Drift**

Standards are needed for documenting training data categories, monitoring model drift, and ensuring policy alignment.

#### **Recommended HHS Role:**

HHS should publish model governance guidance for non-device clinical AI, including recommended oversight committees, documentation standards, and periodic validation expectations.

#### **Upshot:**

1. We have also covered some of the details and our plan in our [Moonshot](#) and [HBR](#) articles.
2. Impact in Prior Authorization/Utilization Management and Payment Integrity Optimization As a Reference Point:
  - 73% reduction in administrative expenses related to prior authorization
  - 85% fewer manual prior authorization decisions
  - 95% reduction in decision time for automated determinations
  - Over 98% accuracy in automated cases

### **4. Promising AI Evaluation Methods (Non-Medical Devices)**

Based on our experience in AI-powered medical review:

#### **Pre-Deployment**

- Retrospective validation against adjudicated cases
- Policy-to-chart traceability testing
- Bias analysis across demographic cohorts

#### **Post-Deployment**

- Continuous accuracy benchmarking
- Human override tracking
- Appeals outcome monitoring
- Model drift detection

HHS should support evaluation pilots through:

- Cooperative agreements
- CMS innovation demonstrations
- Prize competitions focused on burden reduction and explainability

Basys.ai has participated in federal innovation ecosystems, including CMS-related demonstration initiatives (e.g., program integrity innovation challenges), which provide strong testing environments for scalable AI governance models.

**Upshot:** Details outlined [here](#).

### **5. Supporting Private Sector Accreditation and Certification**

HHS can accelerate adoption by:

- Encouraging industry-driven AI accreditation frameworks aligned with CMS program integrity standards
- Supporting independent third-party validation programs
- Developing standardized reporting templates for AI audit logs and documentation

Clear accreditation pathways would reduce procurement uncertainty for health systems and payers.

## 6. Where AI Has Met Expectations and Where It Has Fallen Short

### Met or Exceeded Expectations

- Administrative burden reduction in utilization management
- Faster turnaround times in structured medical review
- Improved consistency in policy application

In CMS-aligned review environments, AI-supported workflows have demonstrated measurable reductions in manual review time while maintaining clinical oversight.

### Fallen Short

- Overreliance on unstructured LLM outputs without guardrails
- Insufficient integration into legacy workflows
- Lack of interoperability with policy databases and EHR systems

### High-Potential AI Areas

- Policy-to-chart matching
- Longitudinal care gap detection
- Early risk stratification in chronic disease management
- Automated fraud, waste, and abuse pattern identification

**Upshot and Learnings:** While there are gaps with commercial insurers, there are models of AI adoption that we can learn from - [basys.ai](https://basys.ai) can help fill that gap.

## 7. Key Decision Makers and Administrative Hurdles

### Primary Influencers

- Chief Medical Officers
- Chief Information Officers
- Utilization Management Leadership
- Compliance and Risk Officers

### Administrative Hurdles

- Lengthy procurement cycles
- Data integration complexity
- Unclear compliance standards
- Limited interoperability across payer-provider ecosystems

From our work with health plans, operational bottlenecks often arise not from model performance but from integration challenges and documentation requirements.

## 8. Interoperability Priorities

Enhanced interoperability would accelerate AI adoption in:

- FHIR-based clinical data exchange
- Structured policy ingestion standards
- Standardized medical necessity criteria encoding
- X12 integration for administrative workflows

Benchmarking tools should include:

- Standardized clinical review datasets
- CMS-aligned performance benchmarks
- Shared de-identified validation corpora

## 9. Patient and Caregiver Perspectives

### Desired Improvements

- Faster coverage determinations
- Reduced paperwork
- Transparent explanations for decisions
- Shorter appeals timelines

### Common Concerns

- Fear of automated denials
- Bias in AI systems
- Lack of human oversight

AI deployment must emphasize transparency, clinician involvement, and accessible explanations to build trust.

At a high level, we deeply care about solving the administrative gaps, and it is personal to the founding of the company as shared by Amber, our CEO, in his TED talk, we are also prepared with the learnings and the implementation guidelines shared across our published learnings in [Harvard Business Review](#), [STAT News](#), and [Forbes](#) articles.

## 10. AI Research Priorities for HHS

### Priority Areas

- AI for administrative burden reduction
- Bias detection and mitigation frameworks
- Explainability methods tailored to clinical-administrative decisions
- Longitudinal population health modeling

### a. Published Findings

Emerging literature suggests AI improves documentation efficiency and triage accuracy when embedded within structured governance frameworks. Administrative AI is less studied and represents an important research opportunity.

### b. Costs, Benefits, and Transfers

Literature often focuses on clinical accuracy but underestimates:

- Administrative cost savings
- Reduced clinician burnout
- Improved patient throughput
- System-wide cost avoidance

HHS can catalyze research measuring system-level efficiency gains, not just predictive performance.

**Upshot:** We have delineated our learnings and a framework [here](#).

## Final Note

Basys.ai maintains distribution partnerships connected to both provider (~90% adoption at full activation) and payer ecosystems (97% adoption at full adoption), which we can promptly galvanize. These relationships can support pilot programs, interoperability testing, and scalable validation efforts if HHS pursues demonstration or cooperative models to accelerate AI adoption in clinical care.

We appreciate HHS's forward-leaning and industry-supportive approach and welcome continued engagement to responsibly accelerate AI deployment in ways that enhance productivity, reduce burden, lower costs, and improve patient outcomes.

#### References:

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2. Amber shares his perspective in Harvard Business Review on using the latest AI technology for reducing administrative burden in healthcare: <https://hbr.org/2025/07/should-your-business-use-a-generalist-or-specialized-ai-model>
3. Arpan Saxena, our CIO, <https://www.forbes.com/councils/forbestechcouncil/2026/02/04/cms-wrote-the-rules-now-can-ai-actually-follow-them/>
4. Basys and Smart Data Solution Partnership Announcement: <https://www.fiercehealthcare.com/sponsored/helping-payers-transform-prior-auth-agent-ai> and <https://www.youtube.com/watch?v=4H0Cujgurk0>
5. Harvard Spotlights basys and Amber Nigam, CEO and a Harvard alum: <https://hsph.harvard.edu/studio/news/alumni-insights-careers-in-entrepreneurship-and-innovation/>
6. Basys and MedeAnalytic Partnership Announcement: <https://finance.yahoo.com/news/medeanalytics-basys-ai-announce-strategic-231500450.html>
7. Basys and RELI Group Partnership Announcement: <https://washingtonexec.com/2025/11/reli-group-partners-with-basys-ai-to-advance-ai-driven-healthcare-integrity/>
8. Dr. Oz's Announcement of [basys.ai](https://www.basys.ai) being chosen as a Finalist for CMS' Chili Cook-Off Challenge: <https://www.linkedin.com/feed/update/urn:li:activity:7399121695711858688/> and <https://www.cms.gov/priorities/crushing-fraud-waste-abuse/overview/crushing-fraud-chili-cook-competition>
9. Amber made the list of top Healthcare CEO's of 2025: <https://thehealthcaretechnologyreport.com/the-top-healthcare-technology-ceos-of-2025/>
10. Healthcare Moonshot Concept: <https://www.beckerspayer.com/payer/scan-group-its-time-for-a-healthtech-moonshot/>
11. STAT News Article on Basys' Founding Story: <https://www.statnews.com/2024/11/13/generative-ai-medicine-health-care-ai-racism/>